

Patient ID: PHolstPeriod: February 22 to April 26, 2024Report Date: April 27, 2024Coverage: 1432/1537h (93%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm
Very High HR > 110 bpm

Low HR < 45 bpm
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm
High Breathing > 30 brpm

High HR > 100 bpm
Very High Breathing > 40 brpm

Patient clinical status over 65 days from Feb 22 to Apr 26

■ HR ■ Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	55 bpm	44 bpm	92 bpm
Breathing	17 brpm	11 brpm	32 brpm

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

86 episodic events Detected, spanning 5 days total. Conditions: Elevated Heart Rate, High Breathing, High Heart Rate, Low Heart Rate, Very High Heart Rate. **Priority grade: Medium**

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
86	124h	Low Heart Rate	1	93%

Major Findings: Sustained low HR (avg 43, min 40)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 23	11 PM to 12 AM	1h	1	Very High Heart Rate	113	111	113	Check temp, hydration, pain; review for arrhythmia or infection
Mar 23	11 PM to 12 AM	1h	1	High Breathing	30	11	38	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Feb 22	5 PM to 6 PM	56h	38	Low Heart Rate	43	40	77	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 10	6 AM to 8 AM	20h	15	Low Heart Rate	43	40	77	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 27	12 AM to 2 AM	5h	4	Low Heart Rate	43	40	68	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 31	5 AM to 8 AM	9h	5	Low Heart Rate	44	40	66	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

+ 2 additional condition(s) across Low Heart Rate; details in monitoring data.

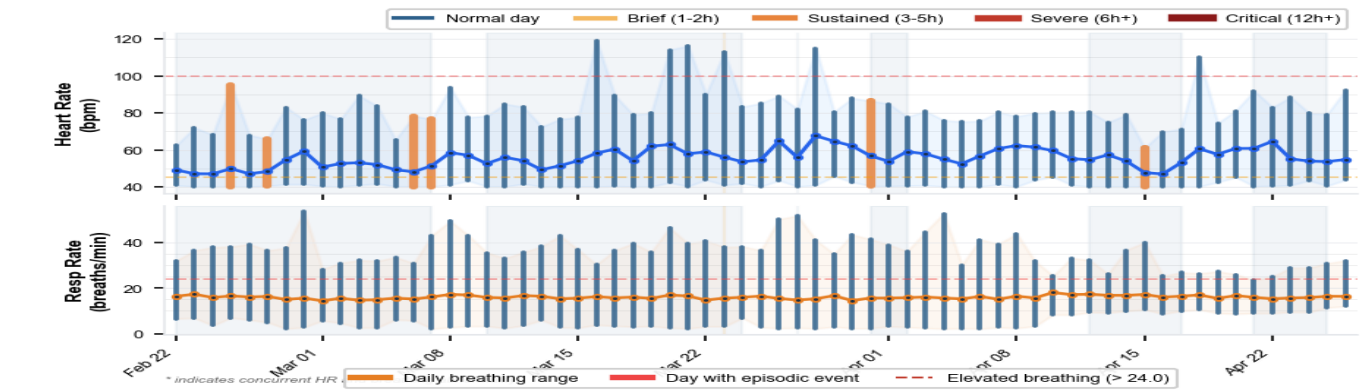
The P5 to P95 heart rate spread was 26 bpm (44 to 70), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 25 brpm, peak 39 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Elevated Heart Rate (avg 98 bpm, peak 115 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- High Heart Rate (avg 106 bpm, peak 119 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- High Breathing (avg 30 brpm, peak 38 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

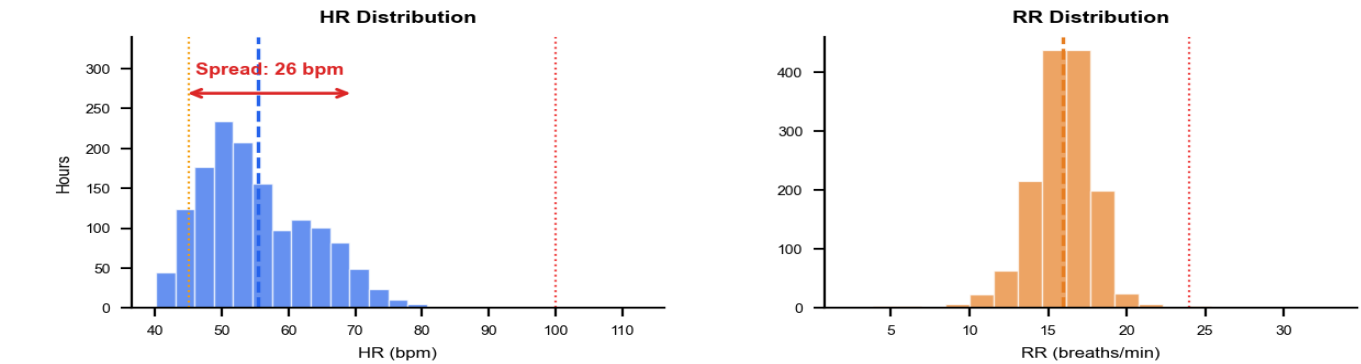
Clinical Pattern Observations:

- 1. **High variability:** HR spread 26 bpm (43 to 70).
- 2. **Clustered pattern:** Episodic events on 6 of 65 days (9%).
- 3. **Nocturnal pattern:** 5 of 6 eligible episodes during evening or night hours.

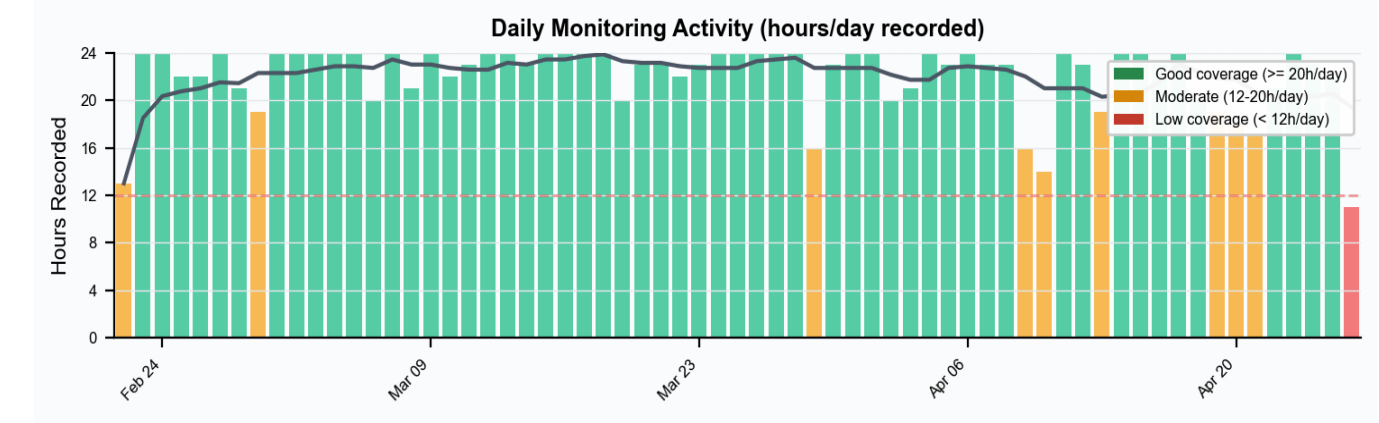


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.